Pivotum

THE DEGREE RISK INDEX · FALL 2026 EDITION

Is Physical Therapy Safe From AI?

The complete profile — the full scores by track, the six factors behind them, the honest downsides, and what to ask before you commit.

2.5

AI EXPOSURE · PHYSICAL THERAPY (HANDS-ON)

where 10 is most at risk

Read this one first; send the student version to them.

HIGHLIGHTING IS COLOUR-CODED

THE FINDING EXPOSURE PROTECTION METHOD & HONESTY

Before you read this — a note for parents

You're likely reading this before your child is. That's normal, and it's worth pausing on how you use it.

Don't lead with the scores. If you open with a risk number, they either get frightened and shut down, or defensive about a decision they've already made.

What this is. A facts document, not a recommendation.

A practical note: there's a separate short version written directly to them. Send them that one.

And one thing specific to this profile, which matters more than anything else here.

The AI question in physical therapy is about as settled as anything in this index. The financial question is not, and it is genuinely serious. The cost of a Doctor of Physical Therapy varies by more than $300,000 between programs offering the identical qualification and leading to the identical career.

That single fact should shape this conversation more than any score. If you read one section of this document, read section 5.

The short answer

Yes — and it is barely a close call.

Hands-on physical therapy scores **2.5** out of 10 for AI exposure — the lowest score of any career in this index apart from surgery. Occupational therapy scores 2.6, speech-language pathology 3.1.

The risk question here is effectively settled. What isn't settled is whether the qualification is worth what it costs — and that depends almost entirely on which program someone attends.

The scores

ROLE	2023	2025	FALL 2026	3-YR MOVEMENT	BAND
Physical therapy (hands-on)	2.0	2.2	**2.5**	+0.5	Very low
Occupational therapy	2.1	2.3	**2.6**	+0.5	Very low
Speech-language pathology	2.6	2.8	**3.1**	+0.5	Low
Respiratory therapy	2.6	2.8	**3.1**	+0.5	Low
Diagnostic imaging technologist	3.5	4.0	**4.4**	+0.9	Low–Moderate
Records / screen-based allied roles	5.1	5.6	**6.1**	+1.0	Moderate–High

Scores run 1–10, *where* 10 *is most at risk. For context: the median profession in this edition sits around* 5.5, *surgery scores* 2.1, *bedside nursing* 2.8, *and entry-level software development* 8.1.

Read the table this way: the +0.5 movement on hands-on therapy is entirely administrative — documentation and exercise-plan drafting. The clinical work has not moved at all.

Why it scores so low — the six factors

Here's how hands-on physical therapy answers them.

HOW MUCH OF THIS JOB CAN AI ALREADY DO? (35% OF THE SCORE)

Less than almost any career we score.

Documentation and progress notes. Exercise plan drafting. Scheduling and billing. Outcome measure scoring. Insurance authorisation paperwork.

What resists: hands on a body, watching a gait in person, feeling resistance and guarding and compensation, motivating someone through months of painful work, and adjusting in real time to what a body actually does.

HOW HARD WILL IT BE TO LAND THAT FIRST JOB? (15%)

Strong protection. Licensure requires supervised clinical hours, which cannot be automated or offshored, and demand is growing well above average as populations age and survival after serious illness improves.

DOES IT HAVE TO BE DONE IN PERSON, WITH YOUR HANDS? (15%)

We rate this 9.5 — the highest physical rating we assign to any career. Section 4 covers both why, and where we think it is most likely to change.

DOES SOMEONE NEED A HUMAN THEY CAN TRUST AND HOLD RESPONSIBLE? (15%)

Yes. Rehabilitation is a relationship over weeks and months, and a licensed clinician carries duty-of-care obligations.

DOES THE LAW REQUIRE A LICENSED HUMAN? (10%)

Comprehensively. Licensure, scope of practice and supervision requirements are written into statute.

HOW OFTEN DOES THE JOB HIT GENUINELY NEW, HIGH-STAKES SITUATIONS? (10%)

Frequently. Every body responds differently, and the judgment about when to push and when to stop is made in the moment.

The protection with the shortest clock

We give physical therapy **9.5** on the physical factor — our highest rating anywhere. It does most of the work in a score of 2.5, and it deserves examining honestly, because of our four protections, physical is the one we rank least durable over a long horizon.

Why it's so strong today. Therapy is manual by definition: hands on a body, resistance felt rather than measured, adjustment in real time to how a person actually moves. Robotics lags software badly, and lags furthest in exactly this setting — an unpredictable human body, in a room, over months.

Why we flag it anyway. A seventeen-year-old entering a doctoral programme will practise for forty years. Over that span, licensure and human trust move at the speed of law and society and are unlikely to shift much. Physical protection moves at the speed of robotics, which is the one thing actively targeting it.

And there is early evidence worth taking seriously. Studies of robotic-assisted rehabilitation report meaningfully better motor outcomes in some domains than conventional therapy — not merely comparable, better. If robotic delivery is superior for certain protocols, "hands-on work is safe" starts looking less like a wall and more like a line.

Two things keep our score where it is. Better outcomes *with* robotic assistance is not the same as therapy delivered *without* a therapist — in every deployment we found, a licensed clinician prescribes, supervises and adapts. And the peer-reviewed literature is explicit that translation into routine practice remains limited.

The specific transition to watch is the same pattern that splits the skilled trades: whether robotic systems move from *assisting* a therapist to *substituting* for therapist hours in structured, protocol-driven rehabilitation. Structured settings are where automation always arrives first. Unstructured ones — a stroke patient at home, an athlete with an atypical injury — are last.

The financial question — and it's the real one

This is the section that should shape the decision, and it has nothing to do with AI.

The debt is substantial and well documented.

- The typical DPT graduate carries roughly **$140,000–$170,000** in total student debt at graduation
- Starting salaries in outpatient settings run **$75,000–$95,000**
- That is a debt-to-first-year-salary ratio of roughly 1.7 to 2.0

For comparison, the same analysis puts new physicians at 0.6–0.9x (much higher debt, much higher salary), nurse practitioners at 0.9–1.3x, pharmacists at 1.5–1.8x, and master's-level occupational therapists at 1.4–1.7x.

The APTA's own research is blunt about the consequences. Asked to give the word they most associate with student debt, physical therapists most frequently answered "*burden*," "*unfair*," "*stressful*" and "*crushing*." Around 28% of PTs report holding additional paid employment alongside their primary position, up from 24% in 2017. Separate industry survey data finds 84% of those with PT debt feel their salary is insufficient to manage the payments.

And here is the number that should decide the program

Analysis across **206 DPT programs at 151 institutions** found debt-to-income ratios ranging from **0.82:1 at the best program to 4.42:1 at the worst.** The median sits at 1.8:1.

That is a difference of more than $305,000 in total cost for the same degree, the same licence and the same career.

Worse, the federal loan cap makes the gap unavoidable at expensive schools. The mean annual funding gap across all 206 programmes is around **$33,647** above the federal borrowing limit — which over three years means roughly $95,000–$101,000 in *private* borrowing at an average-cost school. Private loans carry higher rates and, critically, do not qualify for income-driven repayment or Public Service Loan Forgiveness.

The practical instruction is unusually simple. Attend the least expensive CAPTE-accredited programme you get into. The difference in programme quality is minor relative to the difference in tuition, and the financial consequence lasts decades.

The alternative almost nobody is told about

The physical therapist assistant route.

- **Two-year associate degree** rather than a bachelor's plus a three-year doctorate
- Median pay around **$65,510**, against roughly $80,000–$90,000 for a PT
- Dramatically lower debt — around $24,000 on average where it exists at all

The arithmetic is striking. A PTA working through the years a DPT student spends in graduate school could have earned close to $290,000 by the time the therapist qualifies — while the therapist starts with $140,000–$170,000 of debt.

Over a full career the DPT wins, and comfortably: the income difference compounds across thirty years and the ceiling is considerably higher. But the gap is smaller than families assume, it takes many years to close, and it depends heavily on which programme was attended.

And on our scoring, the AI exposure is similar — PTA work is hands-on, physical and protected by the same things that protect the therapist.

This is not an argument against the DPT. It's an argument that a student drawn to the hands-on work rather than to autonomy and diagnosis should know the assistant route exists before committing to the doctorate.

What the trend shows

Hands-on therapy: 2.0 → 2.2 → 2.5. Up 0.5 in three years — among the slowest movements in the index, and entirely administrative.

Records and screen-based allied roles: 5.1 → 5.6 → 6.1. Up 1.0.

The gap widened from 3.1 points to 3.6. The pattern holds exactly as everywhere else in this index: move the same qualification behind a screen and exposure roughly doubles.

How durable is the protection?

PROTECTION	STRENGTH TODAY	DURABILITY	WHERE THE PRESSURE IS COMING FROM
Physical / embodied	**Highest in the index**	**The one to watch**	Robotic-assisted rehabilitation showing better outcomes in some protocols.
Regulatory / licensing	Strong	**Most durable**	Licensure and scope of practice move at the speed of legislation.
Human trust / accountability	Strong	**Durable**	Rehabilitation is a relationship sustained over months.
Judgment under novelty	Strong	**Durable**	Every body responds differently; the judgment is made in the moment.

The honest read for a seventeen-year-old: they will practise for forty years. Licensure and trust should hold comfortably. Physical protection is the strongest shield today and the one with the most credible long-term threat — and it is worth watching structured, protocol-driven rehabilitation specifically rather than the profession as a whole.

How the role will actually change

WAS THE JOB	IS BECOMING THE JOB
Writing progress notes after every session	Reviewing a generated draft
Building the exercise programme by hand	Adapting a generated programme to this patient
Scoring outcome measures	Reading outcome data that arrives scored
Chasing insurance authorisation	Largely automated
Hands on a body	**Unchanged**
Knowing when to push and when to stop	**Unchanged**

The pattern here is unusually clean. In physical therapy the automatable layer and the clinical layer are cleanly separable, and what's automating is documentation — which the APTA's own research names among the primary contributors to burnout.

Human strengths that will matter most

1. **Manual assessment and treatment** — feeling resistance, guarding and compensation. The

> least automatable thing in this index.

2. **Motivating someone through months of painful work** — most of rehabilitation is adherence,

> and adherence is a relationship.

3. **Real-time judgment** — knowing when to push and when to stop, made in the moment with a

> body in front of you.

4. **Accountability** — carrying duty of care for someone's recovery. 5. **Verification** — knowing when a generated programme or note doesn't fit this patient.

Notably absent: protocol recall and documentation speed. These were traditional markers of a strong new graduate and are the fastest-depreciating.

Other things to consider about this job

What's genuinely good about it

Demand is strong and demographically driven rather than cyclical. Physical therapists are projected to grow around 11% and occupational therapists 14% through 2034 — both far faster than average — driven by an ageing population and improving survival after serious illness.

What therapists report as rewarding is unusually consistent:

- **Visible progress** — you watch someone recover across weeks in a way few jobs offer
- **Relationships that run over months**, not minutes
- **Autonomy**, with private practice a realistic destination
- **Physical, active work** rather than a desk
- **Genuine variety** across settings — hospital, outpatient, sports, paediatric, home health, neurological

And the career is sustainable into later life with specialisation, and portable across settings and geographies.

What's genuinely hard about it

The debt, covered in section 5, and it is the dominant issue.

Burnout is high and the causes are not what you'd expect. Around 70% of physical therapists report moderate to high burnout, and the most-cited drivers are unrealistic productivity demands, heavy documentation requirements and administrative load — not the clinical work itself.

That's worth noticing, because it's precisely the layer that AI is removing. This may be the career in the whole index where automation most directly addresses the profession's actual complaint.

The work is physically demanding over decades. Standing, lifting, transferring patients.

And some patients do not improve, which is harder to sit with than students expect.

Who this work suits — and who it doesn't

Physical therapy tends to suit people who:

- **Want to see progress they can measure** — the single most-cited satisfaction
- **Are comfortable with sustained physical contact** and with bodies generally
- **Can motivate people who don't want to be motivated** — most of the job is adherence
- **Prefer moving to sitting**
- **Are patient with slow improvement**

It's harder for people who:

- Need **quick resolution** — rehabilitation is measured in months
- Find **repetitive documentation** draining, though this is the automating part
- Want **high early earnings** relative to the training cost
- Are uncomfortable when **someone doesn't get better**

What else is moving this market

Demographics are the dominant force, and they are durable rather than cyclical.

Non-opioid pain management has increased referrals to physical therapy meaningfully — a policy shift rather than a technological one.

Productivity pressure is the profession's structural complaint, particularly in outpatient settings where reimbursement models push toward higher patient volumes per therapist.

And employer loan assistance is emerging but rare — around 80% of PTs and OTs carry student debt while only about 8% receive any employer repayment support. Where it exists it is worth several years off a payoff timeline, and it is a legitimate thing to ask about at interview.

The AI-native advantage — what to actually do about it

The situation: AI in physical therapy is arriving as documentation relief rather than clinical substitution, which makes this one of the more straightforwardly positive pictures in the index.

WHAT AN AI-NATIVE THERAPIST LOOKS LIKE

1. Reclaim the time deliberately. Documentation automation only helps if the hours saved go somewhere. Therapists who are intentional about this get more patient contact; those who aren't watch productivity targets absorb it.

2. Verify. Knowing when a generated note or programme doesn't fit this patient — a clinical skill rather than a technical one.

3. Work with robotic and sensor-assisted rehabilitation. These systems are arriving. Understanding where they help and where they mislead is a genuinely new competency and almost nobody is teaching it.

4. Use outcome data properly. Continuous movement and adherence data is becoming available. Interpreting it clinically is a real skill.

5. Contribute to how it's implemented. Services are writing these policies now, largely without practitioner input.

CONCRETE PREPARATION

Before the programme: compare programmes on total cost, not reputation. This is the single highest-value action available and it is worth more than anything else in this section.

During: get clinical exposure across as many settings as possible — the setting shapes the career more than the qualification does. Take manual therapy and assessment seriously; that is the protected core.

Entering: ask about employer loan repayment assistance, and about productivity expectations specifically. Both shape the first five years more than the job title does.

The routes in

ROUTE	TYPICAL LENGTH	NOTES
DPT (Doctor of Physical Therapy)	3 years post-bachelor's	The standard route. Cost varies by over $300,000 between programmes.
Physical therapist assistant	2-year associate	Hands-on work, far lower debt, lower ceiling. Genuinely under-considered.
Occupational therapy	Master's or doctorate	Similar protection, focused on daily function rather than movement.
Speech-language pathology	Master's	Consistently under-recruited, scores 3.1.
Respiratory therapy	Associate or bachelor's	Shorter route, acute-care focused, scores 3.1.
OT assistant	2-year associate	Strong projected growth, short training.

Where these qualifications can lead later

Specialisation (orthopaedic, neurological, paediatric, sports, geriatric), private practice ownership, clinical education and academia, research, healthcare management, ergonomics and occupational health, and industry roles in devices and rehabilitation technology.

The pattern in this index holds: the further from hands-on patient contact, the higher the exposure. Records-centred and utilisation roles score 6.1 against clinical work at 2.5.

What to look for in a program

1. Total cost — and calculate the debt-to-income ratio yourself. This matters more than anything else on this list, and it is the thing prospectuses discuss least.

2. CAPTE accreditation. Non-negotiable.

3. Breadth of clinical placements. How many settings, and which? Setting shapes career satisfaction more than the qualification does.

4. Licensure exam pass rates.

5. Does the curriculum address rehabilitation technology and AI-assisted documentation?

Questions to ask a program

On cost — ask these first:

- What is the total cost of attendance, including living expenses, for the full programme?
- What is the median debt at graduation for your graduates?
- What proportion of students need private loans beyond the federal cap?

On clinical experience:

- How many weeks of clinical placement, across how many distinct settings?
- Are placements guaranteed, and how are they allocated?

On outcomes:

- What's your licensure exam first-time pass rate?
- Where are graduates working at one year, and in what settings?
- What's your attrition rate?

On technology:

- How does the curriculum address robotic-assisted rehabilitation and AI documentation tools?

Red flags: evasiveness about total cost; placements not guaranteed; no data on graduate debt; a curriculum unchanged since 2022.

Go deeper — further reading

- **Physical Therapists — Occupational Outlook Handbook** — *US Bureau of Labor Statistics*. Primary source and free: projected growth, median pay, entry requirements. Read the **Physical Therapist Assistants** page alongside it — the comparison is the most useful twenty minutes available on this decision.

- **Impact of Student Debt on the Physical Therapy Profession** — *American Physical Therapy Association*. The profession's own research into what the debt is doing to its members, including the finding that 28% hold additional employment. Read it before committing to an expensive programme.

- **Is physical therapy worth the debt? 2026 ROI analysis** — the programme-by-programme debt-to-income analysis across 206 DPT programmes. If you read one thing before choosing where to apply, read this.

The counterargument — read this one:

> The strongest case against our 2.5 isn't that AI will replace therapists. It's that a very low AI score may lead a family to under-weight a genuinely serious financial risk.
>
> On this reading, our framework tells a student that physical therapy is among the safest careers available — which is true — while saying nothing about a debt-to-income ratio that can reach 4.42:1 at the wrong programme, private borrowing that doesn't qualify for forgiveness, and 84% of indebted PTs reporting their salary is insufficient to manage the payments.
>
> A career can be automation-proof and still be a poor financial decision.
>
> **Where we land:** this is entirely fair, and it's why section 5 sits before the "other considerations" section rather than inside it. Our score measures automation exposure only. For physical therapy specifically, the AI answer is genuinely reassuring and the financial answer depends almost entirely on which programme someone attends — and conflating the two would be the most damaging thing this document could do.

Bottom line

On AI exposure, this is about as good as it gets. Hands-on physical therapy scores 2.5 — lower than anything in this index except surgery — protected by manual work, licensure, trust and genuine unpredictability. Demand is demographically driven and durable. And the automation that is arriving removes documentation, which the profession's own research names among the leading causes of burnout.

The real decision is financial, and it is program-specific to an extraordinary degree. Debt-to-income ratios across 206 DPT programmes run from 0.82:1 to 4.42:1 — a spread of over $305,000 in total cost for the same degree and the same career.

So the useful advice is unusually concrete: attend the least expensive accredited programme you get into, because the quality difference between programmes is small relative to the cost difference. Minimise undergraduate debt first. Ask employers about loan repayment assistance. And if the pull is hands-on rehabilitation rather than autonomy and diagnosis, look seriously at the assistant route before committing to the doctorate.

And one thing worth saying that no score captures. Watching someone walk again, or lift their arm again, or return to something they thought they'd lost — very few jobs offer that at all. The financial caution above is real and it is about *which programme*, not about *whether*.

Discussion questions

Part 1 — About the work itself

1. What made physical therapy appealing? Push past "I want to help people" to something

> specific — an injury, a person, a moment.

2. Are they comfortable with sustained physical contact, and with bodies generally? 3. Can they motivate someone who doesn't want to be motivated? Most of rehab is adherence. 4. How would they cope with a patient who doesn't improve?

Part 2 — Reacting to the findings

5. Hands-on therapy scores 2.5 and records-based allied work 6.1. Does the setting matter to

> them?

6. The profile flags physical protection as the strongest today and the least durable

> long-term. What do they make of that?

7. Around 70% of PTs report moderate to high burnout, driven by documentation and productivity

> demands rather than clinical work. Did that surprise them?

Part 2b — The money conversation, and this is the important one

8. **Have you calculated the debt-to-income ratio for each programme they're considering?**

> Total cost of attendance divided by realistic starting salary. Do it before applications, not after offers.

9. Do they understand that the same degree costs over $300,000 more at some programmes than

> others?

10. Do they know that borrowing above the federal cap means private loans, which don't qualify

> for income-driven repayment or forgiveness?

11. **Have you looked at the physical therapist assistant route together?** Two years, far less

> debt, similar AI protection, lower ceiling.

Part 2c — The other side

12. Of what therapists find rewarding — visible progress, long relationships, autonomy, physical

> work, variety — which two matter most?

13. Looking at "who this work suits": which items sound like them? Answer separately, then

> compare.

Part 2d — The AI question, practically

14. The automation here is removing documentation, which is the profession's main complaint. Does

> that change how they see the career?

15. Robotic-assisted rehabilitation is showing better outcomes in some protocols. Is that

> interesting or concerning to them?

Part 3 — Testing it against reality

16. **The most valuable single action:** shadow a therapist in two different settings —

> outpatient orthopaedic and something else entirely, like neurological or paediatric. The settings are very different jobs.

17. Find someone five years post-qualification and ask directly about their loan payments and

> whether they'd choose the same programme.

Part 4 — About the program

18. Ask each programme the cost questions from section 14 — first, and before anything else. 19. How many clinical placement settings, and are placements guaranteed? 20. What's the licensure exam first-time pass rate?

Part 5 — Widening the frame

21. Have they considered **occupational therapy, speech-language pathology or respiratory

> therapy**? All score similarly well, and speech-language pathology is consistently under-recruited.

22. What about **nursing**? Scores 2.8, reaches licensed practice in two to four years, and

> costs a fraction as much.

23. If none of these were available, what would they choose? The answer usually reveals what

> they're drawn to.

Part 6 — For the parent, alone

24. Am I comfortable with the debt, and have I said so clearly either way? 25. Have I looked at the actual numbers, or assumed that a doctorate must be worth it? 26. Would I support the assistant route if that's what suited them, or would I quietly see it as

> settling?

27. What's the one thing I most want them to understand — in a sentence, without a number in it?

This is analysis and informed judgment about a fast-moving situation. For physical therapy specifically, we've flagged that the profession's most serious risk is financial rather than technological, and that it varies enormously by programme — something our six factors cannot see.

Scores for 2023 and 2025 are retrospectively calculated using the current methodology. Scores measure exposure to what AI can already do — not how much any particular employer has deployed.

Technical scoring appendix — Physical Therapy & Allied Health

Pivotum Degree Risk Index — Fall 2026 Edition

This appendix is the audit trail. It sets out the formula, the rating anchors, every factor rating behind every track score in this profile, the backcast arithmetic, a sensitivity analysis, and the specific things that would change our mind.

We publish it because a score nobody can check is an opinion with a decimal point.

1. The formula

```
Risk = 0.35 × Automatability
     + 0.15 × EntryErosion
     + 0.15 × (10 − Physical)
     + 0.15 × (10 − Trust)
     + 0.10 × (10 − Regulatory)
     + 0.10 × (10 − Judgment)
```

Two exposure factors carry 50% of the weight; three protection factors carry 40%; judgment under novelty carries the remaining 10% and sits on the protection side. The deliberate consequence is that protection can outweigh exposure — which is why a highly automatable job like teaching still scores low.

Bands. Very low below 3.0 · Low 3.0–4.4 · Moderate 4.5–5.4 · Moderate–High 5.5–6.9 · High 7.0 and above.

2. Rating anchors

HOW THE 0–10 RAW RATINGS ARE ANCHORED

Every factor is rated on its own 0–10 scale before weighting. The anchors are identical across all twenty-seven careers, which is what makes the scores comparable.

Task automatability (A) — *what share of the working week could a capable current system already do to an acceptable standard?*

RATING	ANCHOR
0–2	Almost nothing. The work is physical, relational or wholly novel.
3–4	Administrative surround only — notes, scheduling, correspondence.
5–6	A meaningful minority of the week, mostly documentation and lookup.
7–8	A large share, including tasks central to the junior version of the role.
9–10	Most of the described duties, at or near professional standard.

Entry-path erosion (E) — *how much has the traditional route into this work narrowed?*

RATING	ANCHOR
0–2	Protected by statute or physical necessity. Training hours cannot be automated.
3–4	Stable. Some junior tasks absorbed; the apprenticeship survives.
5–6	Visible narrowing. Employers prefer experience; junior intake reduced.
7–8	Substantial. The work juniors learned on has largely gone.
9–10	The training layer and the automatable layer were the same layer.

Physical / embodied (P) — *must this be done in person, with a body, in an unpredictable setting?*

RATING	ANCHOR
0–2	Fully screen-based. Location-independent.
3–4	Occasional presence; the core work is not physical.
5–6	Regular physical presence, in largely controlled conditions.
7–8	Substantially physical, with some variability of setting.
9–10	Irreducibly physical, in conditions that differ every time.

Human trust / accountability (T) — *does someone need a specific human they can rely on, and must a human answer when it goes wrong?*

RATING	ANCHOR
0–2	Output is anonymous and reviewed by others.
3–4	Work is signed off by someone else. No personal reliance.
5–6	Some client relationship; accountability is institutional.
7–8	Named responsibility, with commercial or professional consequence.
9–10	The relationship is the service, and liability attaches personally.

Regulatory / licensing (R) — *does the law reserve this act to a qualified human?*

RATING	ANCHOR
0–2	No licence exists and none is proposed.
3–4	Certification is customary but not reserved.
5–6	Partial reservation, varying by jurisdiction or task.
7–8	Licensed practice, with some unreserved adjacent work.
9–10	Comprehensive statutory reservation, including of the training route.

Judgment under novelty (J) — *how often does the work present something with no matching precedent, where being wrong is costly?*

RATING	ANCHOR
0–2	Routine by design. Deviation is an error, not a case.
3–4	Occasional exceptions, handled by escalation.
5–6	Regular non-standard cases within a known range.
7–8	Frequent genuine novelty with material consequences.
9–10	Novelty is constant, and in some fields adversarially generated.

3. Factor ratings by track

Ratings are the Fall 2026 edition unless a range is shown. **P, T, R and J are held constant across editions** — those protections are structural and do not move on a three-year horizon. All measured movement is in A and E.

PHYSICAL THERAPY (HANDS-ON) — 2.5 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.1	4.5	**5.2**	1.82
Entry-path erosion	15%	1.2	1.5	**1.8**	0.27
Physical / embodied	15%	9.5	9.5	**9.5**	0.07
Human trust / accountability	15%	9.3	9.3	**9.3**	0.1
Regulatory / licensing	10%	9.3	9.3	**9.3**	0.07
Judgment under novelty	10%	8.5	8.5	**8.5**	0.15
				Total	**2.49 → 2.5**

OCCUPATIONAL THERAPY — 2.6 (VERY LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.3	4.7	**5.5**	1.92
Entry-path erosion	15%	1.2	1.5	**1.8**	0.27
Physical / embodied	15%	9.4	9.4	**9.4**	0.09
Human trust / accountability	15%	9.3	9.3	**9.3**	0.1
Regulatory / licensing	10%	9.3	9.3	**9.3**	0.07
Judgment under novelty	10%	8.5	8.5	**8.5**	0.15
				Total	**2.61 → 2.6**

SPEECH-LANGUAGE PATHOLOGY — 3.1 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	5.2	5.7	**6.4**	2.24
Entry-path erosion	15%	1.5	1.8	**2.0**	0.3
Physical / embodied	15%	8.5	8.5	**8.5**	0.22
Human trust / accountability	15%	9.3	9.3	**9.3**	0.1
Regulatory / licensing	10%	9.3	9.3	**9.3**	0.07
Judgment under novelty	10%	8.5	8.5	**8.5**	0.15
				Total	**3.09 → 3.1**

RESPIRATORY THERAPY — 3.1 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	5.1	5.6	**6.3**	2.2
Entry-path erosion	15%	1.5	1.8	**2.0**	0.3
Physical / embodied	15%	8.8	8.8	**8.8**	0.18
Human trust / accountability	15%	9.0	9.0	**9.0**	0.15
Regulatory / licensing	10%	9.3	9.3	**9.3**	0.07
Judgment under novelty	10%	8.2	8.2	**8.2**	0.18
				Total	**3.08 → 3.1**

DIAGNOSTIC IMAGING TECHNOLOGIST — 4.4 (LOW)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.9	6.1	**7.0**	2.45
Entry-path erosion	15%	2.5	3.0	**3.4**	0.51
Physical / embodied	15%	7.5	7.5	**7.5**	0.38
Human trust / accountability	15%	7.0	7.0	**7.0**	0.45
Regulatory / licensing	10%	8.5	8.5	**8.5**	0.15
Judgment under novelty	10%	5.5	5.5	**5.5**	0.45
				Total	**4.38 → 4.4**

RECORDS / SCREEN-BASED ALLIED ROLES — 6.1 (MODERATE–HIGH)

FACTOR	WEIGHT	2023	2025	2026	CONTRIBUTION TO 2026 SCORE
Task automatability	35%	4.6	5.9	**7.1**	2.48
Entry-path erosion	15%	4.5	5.0	**5.5**	0.82
Physical / embodied	15%	1.5	1.5	**1.5**	1.27
Human trust / accountability	15%	5.5	5.5	**5.5**	0.67
Regulatory / licensing	10%	7.0	7.0	**7.0**	0.3
Judgment under novelty	10%	4.5	4.5	**4.5**	0.55
				Total	**6.11 → 6.1**

4. Backcast arithmetic

Worked example for the headline track, showing exactly how the three-year movement is composed.

Physical therapy (hands-on)

EDITION	A	E	PROTECTION DEFICIT (FIXED)	SCORE
2023	4.1	1.2	0.4	**2.0**
2025	4.5	1.5	0.4	**2.2**
Fall 2026	5.2	1.8	0.4	**2.5**

Movement decomposition: automatability contributed **+0.39** and entry-path erosion **+0.09**, for a total of **+0.5** points over three years.

The 2023 and 2025 figures are retrospective reconstructions using the current methodology, not archived scores from published editions. They are our best assessment of what each factor would have rated at the time, given what was then demonstrable. They are not measurements.

5. Sensitivity

How far the score moves if a single factor is rated one point differently:

FACTOR	±1 POINT MOVES THE SCORE BY
Task automatability	±0.35
Entry-path erosion	±0.15
Physical / embodied	±0.15
Human trust / accountability	±0.15
Regulatory / licensing	±0.10
Judgment under novelty	±0.10

What this means in practice. A one-point disagreement about automatability moves a score by 0.35 — enough to shift a track between bands at the margins. A one-point disagreement about licensing moves it by 0.10, which almost never changes a band.

So if you disagree with one of our numbers, the automatability rating is where the disagreement matters most, and it is the rating we would most want challenged.

6. Stated limitations

We measure capability, not deployment. Scores reflect exposure to what AI can already do at the leading edge — not how much any particular employer has adopted. Adoption lags capability by years and lags unevenly: large, well-funded organisations first; small, rural, public-sector and thin-margin employers much later. The lag is a buffer, not a shelter. It buys time without changing direction.

We do not measure demand. A task can be highly automatable and highly in demand simultaneously. Where the labour market and our framework disagree, we say so in the profile rather than adjusting the score to fit.

We do not measure oversupply. The number of people entering a field is outside our six factors, and for some careers it matters more than automation.

We do not measure industry economics. A profession can contract for reasons entirely unrelated to technology.

We do not measure whether the work is worth doing. Pay, satisfaction, debt, hours and meaning sit outside the score and are covered separately in each profile — often at greater length, because for several careers they matter more.

Sub-track definitions are ours. Job titles vary between employers and countries. We have chosen tracks that reflect genuinely different work, not official classifications.

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7. What would change our mind

Each edition names specific, checkable indicators. If these move, the scores move — including downward.

Across the index:

- **Graduate hiring share.** If employers in the professions where we rate entry-path erosion highly return to hiring juniors at pre-2023 rates, those ratings fall.
- **Content of mandated training hours.** Several professions protect their on-ramp through required supervised experience. If the content of those hours shifts from producing work to supervising it, the protection weakens without any rule changing.
- **Offsite and prefabricated construction share.** The clearest test of our physical-protection ratings: automation performs well in controlled settings, so moving work indoors raises exposure without any robotics breakthrough.

- **Verified deployment rather than capability.** If adoption stalls materially below capability for several editions, our leading-indicator approach is overstating near-term risk and we will say so.

We will report movement in either direction. A framework that only ever revises upward is not measuring anything.

Scores measure exposure to what AI can already do — not how much any particular employer has deployed. The 2023 and 2025 figures are retrospective reconstructions using the current methodology.